

Prior Authorization Request

Health Plan

Humana Medicare Advantage -- Gold Choice PPO.

Member Information

Member: Rosalind M. Fairweather DOB: 1948-09-14 (age 78) Member ID: MBR-13357

Sex: F

Requesting Provider

Practice: Ashgrove Heart Rhythm

Requested Service

Requested service: Elective inpatient admission, implantation of a transvenous implantable cardioverter defibrillator (ICD)

CPT 33249

Planned admit date: 2026-11-02

Level of care requested: INPATIENT

Made-up data for a software prototype. Not a real patient.

EP consultation

ASHGROVE HEART RHYTHM

EP Consultation - 09/21/2026

Pt: Fairweather, Rosalind M. 78F

HPI: NICM dx 08/2025 (cath 08/2025 normal coronaries). On GDMT since 09/2025. Persistent AF since 2024, rate control strategy, failed DCCV x2 (2024, 2025). Dyspnea w/ moderate exertion, walks 2-3 blocks. No syncope.

Data: TTE 09/03/26 LVEF 30%. Holter 09/08-09/10 reviewed.

A/P:

1. NICM, LVEF 30%, NYHA II -> primary prevention ICD.
2. Persistent AF - lenient rate control strategy is acceptable for her (target resting HR <110). Rate control adequate by lenient criteria. No change to AV nodal agents.
3. Will need SDM.

Dr. Maren Ostrowski-Vale

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Holter monitor report

ASHGROVE CARDIAC MONITORING - 48-HOUR HOLTER

Hookup: 09/08/2026 09:12 Duration: 47 h 51 min

Patient: FAIRWEATHER, ROSALIND

Rhythm: atrial fibrillation 100% of recording.

Ventricular rate: min 71 bpm, mean 104 bpm, max 152 bpm (during walking, diary entry 'groceries').

Time with ventricular rate >110 bpm: 41%. Time >130 bpm: 9%.

Nocturnal mean 88 bpm.

Ventricular ectopy: PVCs 1.2%, 3 runs NSVT (max 5 beats).

Pauses: none >2.5 s.

Interpretation: Persistent AF with mean ventricular rate 104 bpm; rates frequently above 110 at rest and with activity. Short runs of NSVT.

Read: H. Yildirim, MD

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Echocardiogram report

ASHGROVE ECHO 09/03/2026

Fairweather, Rosalind

Rhythm during study: AF, HR 98-112

LVEF 30% (biplane, averaged over 5 beats)

Global HK, LV mildly dilated. Severe LA enlargement. Mod MR, mod TR.

Impression: LVEF 30%.

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Heart failure NP note

ASHGROVE HF CLINIC - follow-up 09/28/2026

Rosalind Fairweather

BP 112/70, HR 108 irregularly irregular at rest (seated 5 min).

A: NICM, NYHA II. Persistent AF - rate control borderline; Holter mean 104 with 41% >110. Not sure this counts as adequately controlled for device implant. Defer to EP re: whether to adjust AV nodal therapy before implant. No med change today.

Plan: keep implant date for now.

K. Abernethy-Soto, NP

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Shared decision making note

SDM - ICD - 09/24/2026

Rosalind Fairweather + son (HCPOA)

Clinician: Pilar Encinas, PA-C

Decision aid: evidence-based ICD decision aid (large-print).

Covered benefit, harm, shocks (including inappropriate shocks in AF with fast rates), deactivation, alternatives.

Decision: proceed with ICD.

P. Encinas PA-C

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Medication list

Fairweather, Rosalind - meds 09/28/2026
metoprolol succinate 100 mg daily ... since 09/08/2025
sacubitril/valsartan 24/26 BID since 09/08/2025
spironolactone 12.5 mg since 09/08/2025
empagliflozin 10 mg since 11/2025
digoxin 0.125 mg daily since 2024
apixaban 2.5 mg BID since 2024
furosemide 20 mg daily

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Pre-admission plan

PRE-ADMISSION PLAN - ICD 10/01/2026

Fairweather, Rosalind M. MBR-13357 CPT 33249 Admit 11/02/2026

Expected LOS: 2 midnights.

Factors: age 78; apixaban interruption/restart w/ pocket surveillance; persistent AF on telemetry; digoxin level and K/Mg monitoring.

AF: persistent, rate control strategy per EP (see consult).

No dementia; no terminal illness. No MI, PCI or CABG.

M. Ostrowski-Vale MD

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